Please read carefully!

T.O. Prazosin 1 mg & T.O. Prazosin 2 mg Tablet

DESCRIPTION:

T.O. Prazosin 1 mg Tablet: An orange, round biconvex tablet with a breakline on one side. Each tablet contains prazosin hydrochloride (HCl) 1.095 mg (equivalent to prazosin 1 mg).

T.O. Prazosin 2 mg Tablet: A white round biconvex tablet with a breakline and a figure “2” on one side and the letters “TOC” on the other. Each tablet contains prazosin HCl 2.190 mg (equivalent to prazosin 2 mg).

PHARMACODYNAMICS

Prazosin is an orally active post-synaptic selective alpha adrenoceptor antagonist which reduces peripheral vascular resistance and blood pressure as a result of its vasodilating effects. It produces both arterial and venous dilation.

PHARMACOKINETICS

Prazosin is readily absorbed from the gastrointestinal tract with peak plasma concentrations occurring 1 to 3 hours after an oral dose. The bioavailability is variable and a range of 43 to 85 % has been reported. Prazosin is highly bound to plasma proteins. It is extensively metabolised in the liver, primarily by demethylation and conjugation and is excreted, as metabolites and unchanged prazosin (5 to 11 %), mainly in the faeces via the bile. Less than 10 % is excreted in the urine. Small amounts are distributed into breast milk. Its duration of action is longer than would be predicted from its relatively short plasma half-life of about 2 to 4 hours. Half-life is reported to be increased to about 7 hours in patients with heart failure.

INDICATIONS

Hypertension

T.O. Prazosin 1 mg / 2 mg Tablet is indicated in the treatment of all grades of essential (primary) hypertension and all grades of secondary hypertension of varied aetiology. It can be used as the initial and sole agent or it may be employed in a treatment program in conjunction with a diuretic and/or other antihypertensive drugs, as needed, for proper patient response.

Left Ventricular Failure

T.O. Prazosin 1 mg / 2 mg Tablet is indicated in the treatment of left ventricular failure. Prazosin hydrochloride may be added to the therapeutic regimen in patients who have not shown a satisfactory response or who have become refractory to conventional therapy with diuretics, with or without cardiac glycosides.

Benign Prostatic Hyperplasia

T.O. Prazosin 1 mg / 2 mg Tablet is indicated as an adjunct in the symptomatic treatment of urinary obstruction caused by benign prostatic hyperplasia (BPH). It is also of value in patients awaiting prostatic surgery.

Raynaud’s Phenomenon and Raynaud’s Disease

T.O. Prazosin 1 mg / 2 mg Tablet is indicated in the treatment of Raynaud’s phenomenon and Raynaud’s disease.

RECOMMENDED DOSAGE

T.O. Prazosin 1 mg / 2 mg Tablet is administered orally.

There is evidence that toleration is best when therapy is initiated with a low starting dose of prazosin hydrochloride. During the first week, the dosage of prazosin hydrochloride should be adjusted according to the patient’s individual toleration. Thereafter, the daily dosage is to be adjusted on the basis of the patient’s response. Response is usually seen within 1 to 14 days if it is to occur at any particular dose. When a response is seen, therapy should be continued at that dosage until the degree of response has reached the optimum before the next dose increment is added.

Hypertension

For maximum benefit, small increases should be continued until the desired effect is achieved or a total daily dosage of 20 mg is reached. A diuretic or beta-adrenergic blocking agent may be added to enhance efficacy. The maintenance dosage of prazosin hydrochloride may be given as a twice- or three- times daily regimen.

<u>A. Patients Receiving No Antihypertensive Therapy</u>

It is recommended that therapy be initiated with 0.5 mg given at bedtime, then 0.5 mg two or three times daily for 3 to 7 days. Unless poor toleration suggests that the patient is unusually sensitive, this dosage should be increased to 1 mg given two or three times daily for a further 3 to 7 days. Thereafter, as determined by the patient’s response to the blood pressure lowering effect, the dosage should be increased gradually to a maximum total daily dosage of 20 mg given in divided doses.

<u>B. Patients Receiving Diuretic Therapy with Inadequate Control of Blood Pressure</u>

The diuretic should be reduced to a maintenance dosage level for the particular agent, and prazosin hydrochloride should be initiated with 0.5 mg at bedtime, then proceeding to 0.5 mg two or three times daily. After the initial period of observation, the dosage for prazosin hydrochloride should be gradually increased, as determined by the patient’s response.

<u>C. Patients Receiving Other Antihypertensives but with Inadequate Control</u>

Because some additive effect is anticipated, the dosage level of other agents (e.g., beta-adrenergic blocking agents, methyldopa, reserpine, clonidine, etc.) should be reduced and prazosin hydrochloride initiated at 0.5 mg at bedtime, then proceeding to 0.5 mg two or three times daily. Subsequent dosage increase should be made depending upon the patient's response.

There is evidence that adding prazosin hydrochloride to a beta-adrenergic blocking agent, calcium antagonists or angiotensin-converting enzyme (ACE) inhibitors may bring about a substantial reduction in blood pressure. Thus, the low initial dosage regimen is strongly recommended.

<u>D. Patients with Moderate to Severe Grades of Renal Impairment</u>

Evidence to date shows that prazosin hydrochloride does not further compromise renal function when used in patients with renal impairment. Because some patients in this category have responded to small doses of prazosin hydrochloride, it is recommended that therapy be initiated at 0.5 mg daily and that dosage increases be instituted with caution.

Left Ventricular Failure

The recommended starting dose is 0.5 mg two, three or four times a day. Dosage should be titrated according to the patient’s clinical response, based on careful monitoring of cardiopulmonary signs and symptoms, and when indicated, haemodynamic studies. Dosage titration steps may be performed as often as every 2 or 3 days in patients under close medical supervision.

In severely ill, decompensated patients, rapid dosage titration over 1 to 2 days may be indicated, and is best done when haemodynamic monitoring is available. Adjustment of dosage may be required in the course of prazosin hydrochloride therapy in some patients to maintain optimal clinical improvement.

Benign Prostatic Hyperplasia
The recommended starting dose is 0.5 mg twice daily given for a period of 3 to 7 days and should then be adjusted according to the patient's clinical response. The usual maintenance dose is 2 mg twice daily. The safety and efficacy of a total daily dosage greater than 4 mg has not been established. Therefore, total daily dosages greater than 4 mg should be used with caution.

Raynaud's Phenomenon and Raynaud's Disease
The recommended starting dosage is 0.5 mg twice daily given for a period of 3 to 7 days and should be adjusted according to the patient's clinical response. The usual daily maintenance dosage is 1 mg or 2 mg twice daily. Doses up to 2 mg three times a day may be required for some patients.

Use in Children
Prazosin hydrochloride is not recommended for the treatment of children under the age of 12 years, since safe conditions for its use have not been established.

CONTRAINDICATION
T.O. Prazosin 1 mg / 2 mg Tablet is contraindicated in patients with a known sensitivity to prazosin hydrochloride.

PRECAUTIONS
First dose phenomenon: Prazosin may cause syncope or "first dose phenomenon" characterised by dizziness and faintness, sometimes accompanied by palpitations and fainting.

Syncopal episodes have usually occurred within 30 to 90 minutes of the initial dose; occasionally they have been reported in association with rapid dosage increases or introduction of another antihypertensive drug into the regimen of patient taking high doses of prazosin. Occurrence of this phenomenon is rare at the recommended initial low doses. The dosage should be increased slowly and additional antihypertensive drugs, if necessary, should be introduced into the regimen with caution. The first dose should preferably be taken at bedtime to minimize the possibility of first-dose fainting.

Adrenaline should not be given to antagonize the hypotensive effects of prazosin in overdosage since it may enhance any cardiac accelerating effects which may occur.

T.O. Prazosin 1 mg / 2 mg Tablet should not be used to treat left ventricular failure due to mechanical obstruction such as aortic valve stenosis, mitral valve stenosis, pulmonary embolism and restrictive pericardial disease.

T.O. Prazosin 1 mg / 2 mg Tablet may cause drowsiness or dizziness and hence, patients affected should not drive or operate machinery.

Usage in children:
The safety for use of prazosin in children has not been established.

Usage in pregnancy:
The safety for use of prazosin during pregnancy has not been established. **T.O. Prazosin 1 mg / 2 mg Tablet** should only be used when clearly needed and when the potential benefits outweigh the unknown potential hazards to the foetus.

Usage in nursing mothers:
Since prazosin is distributed into milk in small amount, **T.O. Prazosin 1 mg / 2 mg Tablet** should be used with caution in nursing mothers.

DRUG INTERACTIONS
Addition of a diuretic or other antihypertensive agent to prazosin has been shown to cause an additive hypotensive effect. The risk of first-dose hypotension may be particularly increased in patients receiving beta-blockers or calcium-channel blockers.

Since prazosin is highly protein-bound, **T.O. Prazosin 1 mg / 2 mg Tablet** may theoretically interact with other highly protein-bound drugs.

Laboratory test: Patients treated with prazosin may provide false positive results in screening tests for phaeochromocytoma such as presenting urinary vanillylmandelic acid (VMA) and methoxyhydroxyphenyl glysol (MHPG) (a urinary metabolite of noradrenaline).

SIDE EFFECTS
The side effects noted were mostly subjective side effects which are common in the first phase of all antihypertensive treatment. These effects may diminish with continued therapy or may be relieved by a reduction in dosage.

Common side effects associated with prazosin therapy include dizziness, fatigue, palpitations, headache and nausea. The following side effects, although rare, have also been reported – vomiting, diarrhoea, constipation, abdominal discomfort, oedema, dyspnoea, syncope, tachycardia, orthostatic hypotension, chest pain, nervousness, paraesthesia, skin rashes, urinary frequency, impotence, blurred vision, reddened sclera, tinnitus, dry mouth and nasal congestion.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES
When instituting therapy with an effective antihypertensive agent, the patient should be advised on how to avoid symptoms resulting from postural hypotension and what measures to take should they develop. The patient should be cautioned to avoid situation where injury could result should dizziness or weakness occur during the initiation of prazosin therapy (i.e. driving or operating machinery).

OVERDOSAGE
Symptoms: The symptoms reported for overdosage of prazosin include profound drowsiness, depressed reflexes and/or severely depressed blood pressure.

Treatment: Should overdosage lead to hypotension, support of cardiovascular system is of first importance. The patient should be placed in the supine position in order to facilitate the restoration of blood pressure and normalisation of heart rate. If necessary, vasopressors and/or volume expanders may be used to treat shock. Renal function should be monitored and supported as needed. Prazosin is not removed by dialysis due to the high degree of protein binding.

PACK SIZES
Blister of Alu-PVC of 10 tablets per strip. Box of 10 & 25 strips.

STORAGE CONDITIONS
Do not store above 30°C. Protect from light.
Keep out of the reach of children.
Jauhi daripada kanak-kanak.
Shelf life: Please refer to the outer box label

PRODUCT REGISTRATION HOLDER:
GoodScience Sdn Bhd
No. 7, Jalan KPK 4/3, Kawasan Perindustrian Kundang,
Kundang Jaya, 48020 Rawang,
Selangor, Malaysia.

NAME & ADDRESS OF MANUFACTURER:
Manufactured by:
T.O. Pharma Co. Ltd.
Bangkok 10310, Thailand.

Packed and released by:
GoodScience Sdn Bhd

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